

UEFA ECIS Men Master Extraction

Prepared: 2026-07-27

S200 Master Sheet

- This is the only all-injury ECIS men denominator used in the master extraction.
- Overlapping all-injury, trend, risk-factor, availability, or prognosis papers do not create extra master rows.
- Supplement papers are retained only when they add useful diagnosis/location-specific incidence, burden, time-loss, recurrence, mechanism, or exposure values.

Field	S200 value / master decision
Master source	S200 - Injury rates decreased in men's professional football
Role	Only all-injury ECIS men denominator used in the master extraction
Reporting period	2001/02-2018/19
Population	49 professional elite men's teams; 3302 players
Exposure	1,784,281 player-hours
All injuries	11,820 time-loss injuries
Training / match injuries	5,035 training; 6,785 match
Incidence	6.6 overall; 3.4 training (95% CI 3.3-3.5); 23.8 match (95% CI 23.2-24.4) per 1000 h
Structured parent tissue counts	Muscle injury 4,763; ligament/joint capsule 1,971
Definitions stored here	time-loss injury; incidence per 1000 player-hours; burden as days lost per 1000 player-hours; medical staff reporting

Sparse Visual Source Map

Read this page first: S200 is the master denominator. Papers below either enrich S200 with retained concept rows, are already covered by a stronger retained source, or stay outside the S200 men master.

S200 ECIS MEN MASTER ANCHOR - all-injury denominator, 2001/02-2018/19, 11,820 injuries, 1,784,281 player-hours

Included in ECIS master extraction	Not included: data already captured or superseded	Not included in S200 master: audit-only or separate workspace
S006 - included as LCL + PCL rows	S002 - not included; ACL superseded by S401	S019 - audit-only; risk/prevention association
S007 - included as indirect thigh + direct thigh contusion rows	S011 - not included; common-injury overlap covered by S200 + retained concept rows	S056 - audit-only; COVID special period, illness data incomplete
S043 - included as hamstring row	S042 - not included; hamstring superseded by S043	S057 - audit-only; performance association
S046 - included as hip/groin + adductor rows	S108 - not included; early all-injury data already captured in S200	S079 - audit-only; workload risk factor subset
S091 - included as Achilles tendinopathy + rupture rows	S110 - not included; hip/groin superseded by S046	S084 - audit-only; fixture congestion risk factor
S106 - included as MCL row	S175 - not included; ankle diagnosis detail covered by S107 unless specialist worksheet needed	S199 - audit-only; winter-break availability paper
S107 - included as ankle row	S201 - not included; broad summary superseded by S200	S2391 - audit-only; nested ACL reconstruction prognosis cohort
S113 - included as upper-extremity row	S509 - not included; MCL concept covered by S106	S358 - audit-only; trend/risk paper
S202 - included as stress-fracture row; count-only subtype audit-only	S576 - not included; concussion concept covered by S340	S415 - audit-only; availability/performance paper
S340 - included as head/neck + concussion rows	S581 - not included; RTP prognosis overlaps retained concept rows	S4839 - audit-only; 14-team hamstring burden risk-factor substudy
S368 - included as syndesmosis row		S513 - audit-only; short-turnaround risk factor
S401 - included as ACL row		S527 - audit-only; return-to-play risk paper
S451 - included as fifth-metatarsal fracture row		S554 - audit-only; regional subgroup paper
S5151 - included as a later 2022/23 full-season supplement; S200 remains the anchor		S1091 - duplicate alias of S112; no second WECIS record
		S112 - separate WECIS women master, not part of S200 ECIS men
		S068 - own workspace; South America vs ECIS comparator
		S109 - own workspace; UEFA tournament audit
		S111 - own workspace; UEFA-method paper, non-ECIS/WECIS
		S5338 - separate UEFA EURO tournament workspace, outside ECIS/WECIS

Methodological Justification

Prepared: 2026-07-27

Purpose

- Provide a peer-review-facing rationale for how the UEFA Elite Club Injury Study family was consolidated without duplicate counting.
- Explain why the men ECIS and women WECIS records are treated as separate master records.
- Document why retained supplement rows are field-level enrichments rather than independent all-injury cohorts.
- Document why illness fields remain blank unless a paper reports extractable illness outcomes.

Primary Methodological Principle

- The extraction unit is the surveillance programme-period and directly reported subgroup/concept, not the publication.
- An overlapping paper cannot contribute a second all-injury denominator for the same ECIS or WECIS surveillance population.
- A topic-specific paper can contribute values only for fields that are not already represented by the anchor source and are directly compatible with the live schema.
- This preserves the broad programme denominator while still capturing clinically useful incidence, burden, severity, recurrence, mechanism, and exposure details from companion papers.

Anchor Selection

Master	Anchor	Reason
Men ECIS	S200	Longest and most recent broad all-injury ECIS men denominator in the audited set; used for shared context, definitions, exposure, all-injury counts, and overall incidence.
Women WECIS	S112	Only audited WECIS women all-injury programme paper; kept separate because sex, programme, teams, period, and denominator differ from men ECIS.

Why S200 Is An Anchor, Not A Complete Pattern Table

- S200 is the best ECIS men all-injury denominator anchor, but it does not provide every location and type detail needed for the structured tabs.
- Older broad papers such as early Champions League or 11-year summaries are not imported as ordinary all-injury rows because their periods overlap S200 and their rates would represent shorter historical denominators.
- Those older papers remain available as audit-only historical benchmarks, but they should not be mixed into the main analysis CSV unless a separate historical worksheet is intentionally created.
- The cleaner approach is to keep S200 as the denominator anchor and add retained concept rows from the best companion papers where they report direct additional metrics.

Supplement Inclusion Rule

- Retain a supplement row only when it adds direct incidence, burden, time-loss, mean or total severity days, recurrence, mechanism, or exposure detail beyond the anchor.
- Do not retain count-only subtype rows if the value only subdivides an already retained injury concept without adding analytical metrics.
- Do not back-calculate counts from rates, exposure from counts/rates, or days lost from burden.
- Transparent aggregation is allowed only for directly reported compatible subrows sharing the same denominator and mapping cleanly to a schema parent category.

Structured Location And Type Tabs

- The final structured-tab pass treated injuryTissueType and injuryLocation as metric sweeps, not count-only tabs.
- Where a retained concept row directly reports a compatible count plus overall incidence, burden, mean days, or total days for that same concept, the numeric metric was mirrored into the matching structured row.
- Additional direct structured detail was added from retained ankle and head/neck papers, including ankle pathology incidence/burden/severity and head/neck/concussion/fracture/laceration severity metrics.
- Numeric cells contain numbers only. Confidence intervals, IQRs, caveats, and source logic are kept in outcome fields, source quotes, PDF/audit notes, or backlog text rather than inside numeric boxes.

Illness Decision

- The ECIS and WECIS anchor papers focus on injuries; illness is mentioned mainly for availability, attendance, COVID context, or surveillance-method references.
- The COVID restart ECIS paper was checked specifically; it states that COVID illness data were incomplete because there was no accompanying illness card.
- The head/neck ECIS paper explicitly reports that no illnesses were recorded.
- The non-ECIS UEFA-model NWPL paper mentions an illness card but does not provide extractable illness count, incidence, burden, region, etiology, or severity tables in the reviewed text.
- Therefore illness tabs remain blank in the ECIS/WECIS masters unless a future source reports direct illness outcomes with a compatible denominator.

CSV And Analysis Implications

- Use the master records for ECIS/WECIS analytical export, not the ordinary overlapping UEFA source records as separate all-injury records.
- Keep retained supplement rows identifiable by sex/scope labels such as male - hamstring-specific study or male - ACL-specific study.
- When comparing with CSV exports, filter or flag source-role values so anchor rows, retained concept rows, audit-only sources, and separate-workspace UEFA-related records are not pooled incorrectly.
- If a future analysis needs historical broad location/type distributions, create a separate historical benchmark worksheet instead of merging those values into the S200 master denominator.

Residual Limitations

- The approach prioritizes avoiding duplicate denominators over maximizing the number of filled cells.
- Some older broad location/type tables are intentionally absent from the live master because they would represent shorter or different denominator frames.
- Some parent structured fields remain blank when papers report subrows that do not cover the full parent category or when burden is a median/IQR summary rather than an additive raw component.
- These blanks are methodological safeguards, not missed extraction opportunities.

Live ECIS Master Rows

Source	Live row	Reporting period	Information retained
S200	All injuries anchor	2001/02-2018/19	Base denominator plus shared definitions/context
S043	Hamstring injuries	2001/02-2021/22	Incidence, time-loss, recurrence, exposure
S046	Hip/groin injuries	2001/02-2015/16	Incidence, burden, time-loss, mechanism, recurrence, exposure
S046	Adductor-related hip/groin injuries	2001/02-2015/16	Diagnosis-specific incidence/counts
S006	LCL injuries	2001-2018	Incidence, burden, time-loss, mechanism, recurrence, exposure
S006	PCL injuries	2001-2018	Incidence, burden, time-loss, mechanism, recurrence, exposure
S106	MCL injuries	2001/02-2011/12	Incidence, burden, time-loss, mechanism, recurrence, exposure
S401	ACL injuries	2001-2015	Incidence, recurrence/severity, exposure
S107	Ankle injuries	2001/02-2011/12	Incidence, time-loss, recurrence, exposure
S368	Isolated syndesmotic ankle injuries	2001-2016	Incidence, burden, total time-loss, recurrence, exposure
S113	Upper extremity injuries	2001-2011	Incidence, time-loss, recurrence, exposure
S340	Head/neck injuries	2001/02-2009/10	Incidence, time-loss, exposure
S340	Concussion	2001/02-2009/10	Diagnosis-specific incidence/time-loss
S202	Stress fractures	ECIS-related cohorts	Incidence and recurrence-rate detail
S451	Fifth metatarsal fractures	2001-2012	Incidence, recurrence/non-contact, exposure
S091	Achilles tendinopathy	2001-2011	Incidence, burden, time-loss, recurrence, exposure
S091	Achilles tendon rupture	2001-2011	Incidence, burden, time-loss, exposure
S007	Indirect thigh muscle injuries	2001-2013	Incidence, burden, time-loss, mechanism, exposure
S007	Direct thigh muscle contusions	2001-2013	Incidence, burden, time-loss, mechanism, exposure
S5151	2022/23 World Cup-season all injuries	2022/23	Full-season exposure, count, training/match incidence, total time-loss

Included Source Decisions

Study	Year	Tag	Topic	Period	How included	Reason
S006		included here	LCL/PCL injuries	2001-2018	Live ECIS rows: LCL injuries; PCL injuries.	Adds diagnosis-specific incidence, burden, time-loss, recurrence/mechanism, and exposure not isolated in S200.
S007	2015	included here	Indirect thigh strains and direct thigh contusions	2001-2013	Live ECIS rows: indirect thigh muscle injuries; direct thigh muscle contusions.	Adds directly reported incidence, burden, time-loss, mechanism, and exposure for thigh injury subtypes.
S043	2022	included here	Hamstring injuries	2001/02-2021/22	Live ECIS row: hamstring injuries. Structural/functional subtype count rows remain audit-only.	Adds longer hamstring-specific match/training incidence, time-loss, recurrence, and exposure. Subtypes are not separate master rows because they mainly add counts/time-loss without compatible incidence/burden.
S046	2019	included here	Hip/groin and adductor-related injuries	2001/02-2015/16	Live ECIS rows: hip/groin injuries; adductor-related hip/groin injuries.	Adds incidence, burden, time-loss, mechanism, recurrence, and exposure for a location/diagnosis not isolated in S200.
S091	2013	included here	Achilles tendon injuries	2001-2011	Live ECIS rows: Achilles tendinopathy; Achilles tendon rupture.	Adds diagnosis-specific incidence, burden, time-loss, recurrence, and exposure.
S106	2013	included here	MCL injuries	2001/02-2011/12	Live ECIS row: MCL injuries.	Adds diagnosis-specific incidence, burden, time-loss, recurrence/mechanism, and exposure.
S107	2013	included here	Ankle injuries	2001/02-2011/12	Live ECIS row: ankle injuries.	Adds ankle-specific incidence, time-loss, recurrence, exposure, and structured location/tissue values.
S113	2013	included here	Upper extremity injuries	2001-2011	Live ECIS row: upper extremity injuries.	Adds incidence, time-loss, recurrence, and exposure for a location group not isolated in S200.
S200	2021	master sheet	All injuries, men's ECIS	2001/02-2018/19	Top live ECIS row and master denominator.	Only all-injury ECIS men denominator used for the master; shared definitions and cohort context are stored here.
S202	2012	included here / partial audit-only	Stress fractures	ECIS-related cohorts	Live ECIS row: stress fractures. Fifth-metatarsal stress-fracture count-only detail is audit-only.	Overall stress-fracture incidence and recurrence-rate detail are retained; count-only subtype detail is not a separate master row.
S340	2013	included here	Head/neck injuries and concussion	2001/02-2009/10	Live ECIS rows: head/neck injuries; concussion.	Adds location/diagnosis-specific incidence, time-loss, and exposure.
S368	2019	included here	Isolated syndesmotic ankle injuries	2001-2016	Live ECIS row: isolated syndesmotic ankle injuries.	Adds directly reported incidence, burden, total time-loss, recurrence, and exposure.
S401	2016	included here	ACL injuries	2001-2015	Live ECIS row: ACL injuries.	Retained as the strongest ACL-specific ECIS row with direct incidence, recurrence/severity, and exposure.
S451	2013	included here	Fifth metatarsal fractures	2001-2012	Live ECIS row: fifth metatarsal fractures.	Adds fracture-specific incidence, recurrence/non-contact detail, and exposure; separate from stress-fracture case definition.
S5151	2025	included supplement	2022/23 World Cup-season injury incidence	2022/23	Live ECIS row: 2022/23 World Cup-season all injuries.	Adds a later non-overlapping full-season ECIS denominator, injury count, training/match incidence, and directly reported total absence days while preserving S200 as the anchor.

Already Included, Superseded, Or Audit-Only

- Already included means the S200 master row already captures the relevant all-injury denominator or broad ECIS programme information.
- Superseded means a stronger or longer ECIS source is the retained live row for the same concept.
- Audit-only means the paper may be useful for interpretation, risk/prognosis, subgroup, trend, or future specialist worksheets but should not add rows to this master.

Study	Year	Tag	Topic	Period	Handling	Reason
S002	2011	superseded	ACL injury	earlier ECIS/cohort period	Do not add a live row; use S401 ACL as the retained ACL concept row.	Older ACL-focused source overlaps the ECIS programme and is superseded for master extraction by the later 15-year ACL paper.
S011		already included / audit-only	Common injury patterns / prevention-era benchmark	overlapping 11-year ECIS period	No live ECIS row unless a reviewer requests a historical common-injury benchmark worksheet.	All-injury denominator overlaps S200; common diagnosis rows would duplicate concepts already represented by cleaner retained concept papers.
S019	2020	audit-only	Team injury burden / prevention practice association	overlapping ECIS period	Do not add to master rows; extract in own workspace only if team-level risk-factor analysis is needed.	Association/risk-factor paper, not a clean additional injury-count denominator for the ECIS master.
S042	2016	superseded	Hamstring time trend	2001/02-2014/15	No live ECIS row; use S043 hamstring row.	Earlier hamstring time-trend paper is superseded by the longer 21-season hamstring paper retained in the master.
S056	2022	audit-only special period	COVID-19 lockdown/restart injury burden	COVID restart period	Do not add to the master denominator; extract separately only for COVID-specific analysis.	Special-period subgroup analysis would not be combinable with S200 programme-period counts.
S057	2013	audit-only	Team performance impact	overlapping 11-year ECIS period	No live ECIS row.	Performance association paper; injury denominator overlaps the ECIS all-injury programme already represented by S200.
S079	2018	audit-only risk factor	Internal workload and non-contact injury	one ECIS season / five teams	No live ECIS row unless a separate workload-risk worksheet is created.	Risk-factor analysis with a restricted cohort; not a clean programme-wide count to merge into the master.
S084	2013	audit-only risk factor	Fixture congestion and muscle injuries	overlapping ECIS period	No live ECIS row.	Exposure/risk-factor paper; muscle injury parent counts are already represented by S200 and specific thigh/hamstring papers.
S108	2005	already included	Early Champions League prospective injury study	early ECIS/Champions League period	No live ECIS row.	Early all-injury surveillance is incorporated into the broader S200 ECIS master period.
S110	2009	superseded	Hip/groin injuries	earlier ECIS hip/groin period	No live ECIS row; use S046 hip/groin rows.	Earlier hip/groin source is superseded by the longer retained hip/groin paper.
S175	2022	covered by ankle row / audit-only	Anterior ankle impingement	overlapping ECIS ankle period	No live ECIS row unless a reviewer requests a diagnosis-level ankle worksheet.	Specific ankle diagnosis overlaps the retained S107 ankle row and appears more useful for diagnosis detail than for master-level incidence/burden consolidation.
S199	2019	audit-only availability paper	Winter break and player availability	overlapping ECIS period	No live ECIS row.	Availability/performance association paper; does not add a clean master injury-count denominator.
S201	2008	already included / superseded	General epidemiology of football injuries	earlier/summary source	No live ECIS row.	General epidemiology source is superseded for master extraction by S200 and retained diagnosis-specific papers.
S2391	2025	audit-only	Thigh muscle injury after ACL reconstruction	nested ECIS cohort, 2001-2022	No live master row; use a separate prognosis workspace if required.	The 110 ACL reconstruction cases are nested within the ECIS cohort, so pre/post return-to-play incidence rows would double-count the programme and mix prognosis with surveillance.

Study	Year	Tag	Topic	Period	Handling	Reason
S358	2013	audit-only trend paper	Lower injury rates / prevention or time-trend analysis	overlapping ECIS period	No live ECIS row unless a trend-specific worksheet is created.	Trend/risk paper overlaps the S200 denominator and is not a separate master population.
S415	2013	audit-only availability paper	Match congestion and performance/player availability	overlapping ECIS period	No live ECIS row.	Team availability/performance association paper; not a clean additional injury outcome row.
S4839	2025	audit-only	Medical/performance staff communication and hamstring burden	14-team substudy, 2019/20-2022/23	No live master row; use a separate risk-factor workspace if required.	The source reports team-season burden and predictor associations rather than a pooled programme-wide hamstring incidence or burden compatible with the S043 master row.
S509	2019	duplicate/superseded	MCL injuries	overlapping MCL period	No additional live row; use retained S106 MCL row.	Appears to duplicate or overlap the MCL concept already represented by S106.
S513	2018	audit-only risk factor	Short turnaround and muscle injury rate	overlapping ECIS period	No live ECIS row.	Fixture-congestion/risk-factor analysis; not a separate master denominator.
S527	2020	audit-only prognosis	First match after return to play	overlapping ECIS RTP period	No live ECIS row.	Return-to-play risk/prognosis source rather than a clean incidence/burden extraction row.
S554	2013	audit-only subgroup paper	Regional differences in injury incidence	overlapping ECIS period	No live ECIS row unless a region-specific subgroup worksheet is created.	Regional subgroup incidence would split the same ECIS denominator already held by S200.
S576	2014	covered by concussion row / audit-only	Concussion and subsequent injury risk	overlapping ECIS concussion period	No additional live ECIS row; use S340 for concussion incidence/time-loss.	Prognosis/risk paper; concussion extraction concept is already represented by S340.
S581	2020	audit-only prognosis	Time before return to play for common injuries	2001-2017	No live ECIS row.	Index-injury RTP rows duplicate diagnosis concepts already represented by cleaner incidence/burden source papers.

Not Part Of ECIS Men Master

- These records are UEFA-related but should not be merged into the ECIS men master.

- If eligible, they stay in their own workspace or in the WECIS master, not inside the S200 men master.

Study ID	Year	Status	Category	Role	Decision	Notes	Title
S1091	2024	uefa	WECIS women	duplicate alias	Do not create another WECIS extraction or master row; retain S112 as source-of-truth.	The title, DOI, article pagination, 1,527 injuries, 463 injured players, and Table 2 values match S112. The PDF hash differs only because it is a separate downloaded rendition.	UEFA Women's Elite Club Injury Study: a prospective study on 1527 injuries over four consecutive seasons 2018/2019 to 2021/2022 reveals thigh muscle injuries to be most common and ACL injuries most burdensome.
S112	2024	extracted	WECIS women	master anchor	Use as the WECIS women all-injury source.	Separate master because sex, programme start, and surveillance period differ from men ECIS.	Hallen A. and Tomas R. and Ekstrand J. and Bengtsson H. and Van den Steen E. and Hagglund M. and Walden M. AO - Hallen et al. - 2024 - UEFA Women's Elite Club Injury Study a prospective study on 1527 injuries over four consecutive sea.pdf
S068	2021	extracted	UEFA-related comparator	own workspace	Extract in its own paper workspace if needed; do not create a master record.	Contains a South American cohort plus ECIS European comparator; independent South American data must not be folded into ECIS.	Injury epidemiology in professional football in South America compared with Europe
S109	2009	extracted	UEFA tournament	own workspace	Extract in its own paper workspace if needed; do not create a master record.	Tournament audit is UEFA-branded but not ECIS/WECIS elite-club longitudinal surveillance.	Hagglund M. and Walden M. and Ekstrand J. - 2009 - UEFA injury study - An injury audit of European Championships 2006 to 2008.pdf
S111	2019	extracted	UEFA-method-only/non-E CIS	own workspace	Extract in its own paper workspace if it is eligible; do not create a master record.	Uses UEFA methodology/model language but does not appear to be ECIS/WECIS surveillance.	Ibikunle - 2019 - UEFA model in identification of types, severity and mechanism of injuries among footballers in the N.pdf
S5338	2025	extracted	UEFA tournament	separate workspace	Keep outside UEFA-ECIS-MASTER and WECIS; extract only in its own tournament workspace if eligible.	UEFA branding does not make a tournament cohort part of the longitudinal elite-club ECIS/WECIS programme.	Injury incidence and patterns in men's UEFA EURO 2024 and CONMEBOL Copa America 2024: descriptive findings from two simultaneously staged tournaments.

Reviewer Checklist

- Confirm S200 remains the only all-injury ECIS men denominator.
- Confirm every retained supplement row has a useful metric beyond count-only overlap.
- Confirm every non-retained paper has a visible tag: already included, superseded, audit-only, or separate workspace/WECIS.
- Confirm no ordinary UEFA source paper is exported analytically alongside the master in a way that double counts ECIS data.